

HCP Digital Profiling for an Oncology Medical Device Launch

A mixed-method study of professional networks, information sources, clinical priorities, and attitudes toward innovation.

REPORT OVERVIEW

This report shows how public digital signals and qualitative review were combined to create a more useful picture of a specialized oncology audience. The work supported launch planning for a medical device while protecting the identity of the client, product, and individual participants.

106	16	14
profiles in the quantitative audience	physicians in qualitative review	detailed clinician profiles

Study type	Scope	Research period
Digital audience profiling + qualitative review	US oncology and breast-care professionals	Historical case based on 2022 research

Prepared by HealthLife Metrics

Research and analytics for healthcare decisions

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A practical account of the research design, findings, and implications.

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How to read the evidence. Quantitative charts show platform-estimated attributes or affinities within the selected digital audience. Qualitative findings summarize recurring themes in public clinical footprints. Neither should be interpreted as a representative survey of all oncologists.

From scattered digital traces to a launch-ready audience view.

The business problem was straightforward: a healthcare team preparing to launch an oncology-focused medical device needed to understand which clinicians mattered, where they participated professionally, what information environments surrounded them, and how their clinical priorities differed. A basic contact list could identify names. It could not explain context.

The research therefore combined audience discovery, manual validation, relationship mapping, platform-based audience analysis, and qualitative review of public professional materials. This created two complementary levels of evidence. The quantitative layer showed broad patterns across 106 profiles. The qualitative layer examined 16 physicians in greater depth and produced 14 detailed profiles.

The result was not a ranking of “best” clinicians. It was a structured view of a heterogeneous audience that could support smarter recruiting, content development, channel planning, and message testing.

Core outputs

- Audience profile: estimated demographics, geographic signals, interests, and professional affinities.
- Network view: relationships among clinicians, specialties, institutions, and professional organizations.
- Information ecosystem: influential people, societies, journals, institutions, events, and media sources.
- Professional segmentation: large clinical centers, clinicians with major academic publications, and small or midsize hospitals.
- Qualitative personas: recurring differences in attitudes toward innovation, evidence, patient experience, and implementation.

The central finding was that relevance depended on context. Different clinicians could be interested in the same clinical problem while requiring different evidence, language, and implementation support.

A specialized audience could not be treated as one market segment.

The launch required an evidence base for decisions that cut across research, marketing, medical communication, and outreach. Public information about oncology professionals existed, but it was fragmented across institutional biographies, publication histories, interviews, conference participation, social profiles, and professional networks.

A useful research design had to solve three problems at once. First, it had to identify relevant professionals without assuming that visibility equaled influence. Second, it had to organize a complex network without forcing clinicians into artificial, exclusive categories. Third, it had to translate digital evidence into practical questions for launch planning.

Research objectives

Objective	Decision supported
Identify relevant oncology and breast-care professionals	Define a credible audience for analysis and future research recruitment.
Map professional affinity and influence	Understand the people and organizations that structure the information environment.
Compare interests and information sources	Identify content territories and potential channels for validation.
Segment professional contexts	Adapt questions and materials to different institutional and academic settings.
Explore clinical attitudes and evidence needs	Create useful hypotheses for concept and message testing.

The research was designed to reduce uncertainty before activation. It did not attempt to predict individual prescribing, purchasing, or adoption behavior.

A staged process added context as the audience narrowed.

The study used a funnel-shaped design. Broad digital discovery created the initial universe. Manual review removed irrelevant or ambiguous profiles. Network and audience analysis then identified patterns at scale. Finally, qualitative review added clinical and professional context that could not be inferred from platform data alone.

Stage	What was done	Primary output
1. Discovery	Scraping and API-supported searches identified public profiles connected to oncology and breast care.	Candidate profile universe
2. Manual validation	Profiles were reviewed for professional relevance and fit with the study scope.	Validated audience
3. Audience analysis	A selected group of 106 profiles was analyzed using platform-based audience tools.	Estimated attributes, interests, and affinities
4. Network mapping	Following and follower relationships were used to visualize professional connections.	Affinity and influence maps
5. Qualitative review	Publications, interviews, congress materials, institutional pages, and other public footprints were reviewed for 16 physicians.	Clinical themes and personas
6. Detailed profiling	Fourteen clinicians were summarized in structured individual profiles.	Context for outreach and future research

Why use mixed methods?

Digital audience analysis is efficient for detecting broad patterns, but its estimates can be opaque and incomplete. Manual and qualitative work makes those signals interpretable. Conversely, qualitative profiles offer depth but cannot describe prevalence. Using the methods together allowed the team to distinguish between observed public evidence, platform estimates, and researcher interpretation.

Sources, sample, and analytical boundaries.

The project drew on publicly available professional material and platform-generated audience estimates. No private clinical records or patient-level data were used. The audience was deliberately focused and should be understood as a strategic research sample rather than a statistically representative sample of all oncology professionals.

Source categories

Source category	Examples of evidence	How it was used
Professional profiles	Institutional biographies, public social profiles, specialty information	Identity validation and professional context
Academic record	Publications, journal presence, research topics	Research orientation and active subject matter
Professional networks	Following and follower relationships, society connections	Affinity and influence mapping
Public discourse	Interviews, webinars, conference content, online statements	Clinical priorities, language, and attitudes
Platform estimates	Age, location, interests, media and brand affinities	Directional audience-level comparison

Sample structure

106	16	14
quantitative profiles	qualitative reviews	detailed profiles

Interpretation rule: a digital affinity means that the platform estimated a connection or interest. It does not prove that a clinician read, attended, endorsed, purchased, or adopted anything.

The digital audience showed a strong scientific and health orientation.

The selected profiles were analyzed as a group to identify directional patterns in demographics, location, and interests. The platform described the audience as US-based female clinicians, with the largest estimated age concentration between 35 and 44. Cleveland appeared as the strongest single location signal in the original output, although the audience was distributed across multiple cities.



Figure 1. Platform-estimated age distribution. The largest modeled group was ages 35-44 (36.87%), followed by ages 45-54 (28.90%).



Figure 2. Platform-estimated location signals. Cleveland represented 9.52%; several other cities each represented 4.76%.

These attributes helped describe the analyzed profiles, but they were not used as proof that the wider market had the same composition. Their practical role was to support recruitment planning and to identify where additional validation might be valuable.

Interest estimates reinforced the need for evidence-led communication.

Health and fitness and science were the strongest modeled interests. Technology and computing also appeared above the platform benchmark for the US general population. The comparison suggested an audience likely to encounter product communication within a broader evidence and innovation environment, rather than through consumer health language alone.

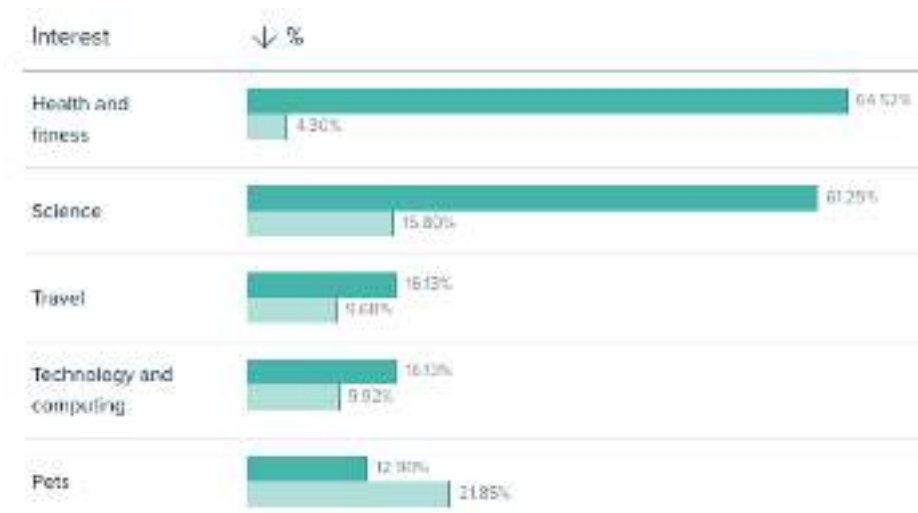


Figure 3. Selected modeled interests compared with the US general population. Percentages are platform estimates.

Interest	Analyzed audience	US general population	Directional reading
Health and fitness	64.52%	4.30%	Very strong modeled concentration
Science	61.29%	15.80%	Strong modeled concentration
Travel	16.13%	9.68%	Moderately elevated
Technology and computing	16.13%	9.92%	Moderately elevated
Pets	12.90%	21.85%	Below benchmark

Strategic reading

The interest profile supported a hypothesis that launch materials should lead with credible clinical evidence and practical usefulness. It did not establish which message would perform best. That question would require direct concept testing or observed campaign data.

Four overlapping professional ecosystems structured the audience.

The network map grouped visible relationships into four broad affinity contexts: breast surgery, plastic surgery, plastic and reconstructive practice, and general medicine or center-based care. The clusters were not treated as exclusive segments. A clinician could sit between contexts or participate in more than one professional community.

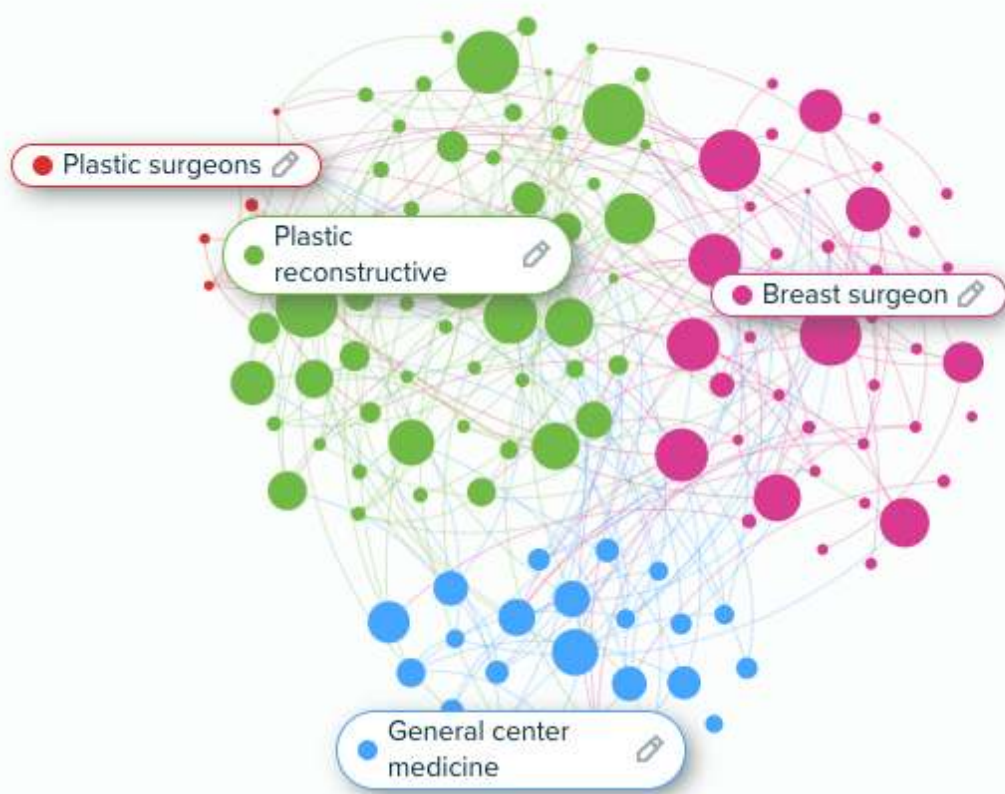


Figure 4. Professional affinity network generated from following and follower relationships. Node position and proximity are analytical signals, not rankings.

Why the network mattered

The map showed why a single launch narrative could miss important differences. A breast-surgery audience may prioritize margin assessment and re-excision. A reconstructive context may place greater emphasis on tissue preservation, aesthetics, and workflow coordination. Center-based clinical environments may introduce additional institutional stakeholders and implementation requirements. These are hypotheses for research and planning, not conclusions about every clinician in a cluster.

Specialist voices and professional societies provided a credible information layer.

The affinity analysis surfaced organizations and individuals connected to surgery, breast care, and oncology. Professional societies included the American Society of Breast Surgeons, the American College of Surgeons, the Society of Surgical Oncology, and the American Society of Clinical Oncology. Specialist voices included clinicians such as Deanna Attai, Judy Boughey, J. Dietz, and Stephanie Valente.

Name	Biography	Affinity
 ASBrS	The American Society of Breast Surgeons, the primary leadership organization for general surgeons who treat patients with breast disease.	88.98% 4.47%
 American College of Surgeons	Highest standards, better outcomes. We are the largest professional organization of surgeons in the world. #ToHeadIt	82.27% 4.21%
 Dr. Deanna Attai	@DrDeannaAttai Assoc Prof, @ASBrS past prez, Fellow @AmColSurgeons, gardener, cat mom	80.58% 4.03%
 Society of Surgical Oncology	The mission of the Society of Surgical Oncology is to improve patient care by advancing the science and practice of surgical oncology worldwide. #SSO	55.00% 3.71%
 Annals Surg Oncology	The Leading Resource in Surgical Oncology	50.77% 3.55%
 Judy Boughey	Breast Surgeon at Mayo Clinic, Professor of Surgery, Chair - Division of Breast and Melanoma Surgical Oncology	50.77% 3.55%
 ASCO	Account of the American Society of Clinical Oncology & its affiliate the Association for Clinical Oncology. Assoc tweets related #ASCOAdvocacy, RTs @endorse.	50.77% 3.55%
 Jill Dietz	Breast Surgeon, Mom, Cancer Fighter and Advocate for High Value and Patient-centric Care.	50.00% 3.32%
 Stephanie Valente	#BreastCancer Surgeon, Director West Region, Fairview Hospital Breast Surgery and Breast Surgical Oncology fellowships @ClevelandClinic #ClevelandCancer	41.89% 3.48%

Figure 5. Selected influencer and organization affinity signals from the source analysis.

The finding did not show endorsement or prove that the audience consistently consumed content from every listed source. It provided a practical starting point for channel research: examine where credible clinical conversations already occur, then validate reach and response with direct performance data.

Institutions and journals anchored the scientific environment.

The institutional landscape included major cancer centers and academic medical organizations, including Dana-Farber Cancer Institute, MD Anderson Cancer Center, Memorial Sloan Kettering Cancer Center, Cleveland Clinic, Mayo Clinic, and Johns Hopkins. Publication affinities included high-authority medical and scientific journals such as the New England Journal of Medicine, JAMA, The Lancet, The Lancet Oncology, and BMJ.

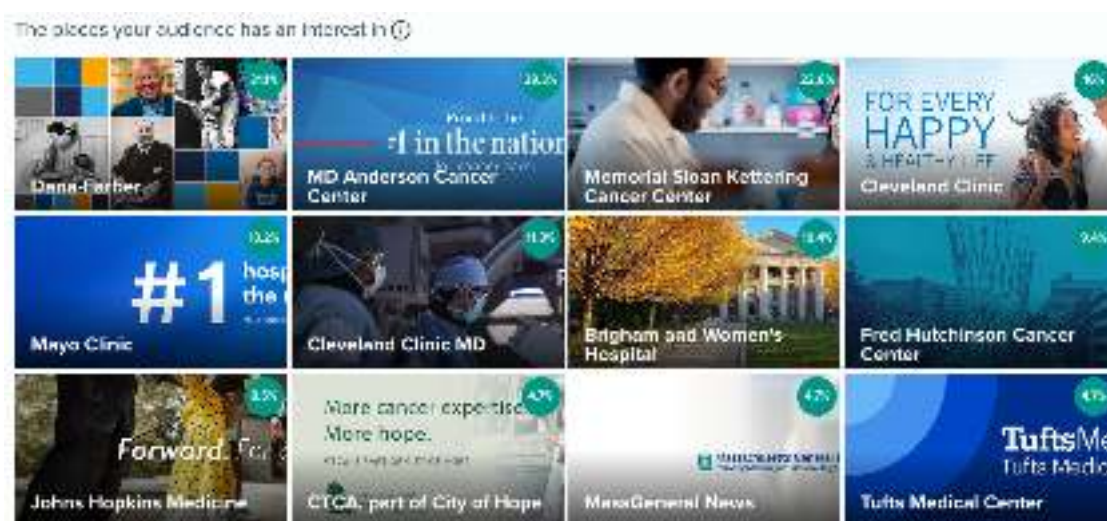


Figure 6. Selected institution and place affinities in the analyzed audience.



Figure 7. Selected magazine and publication affinities in the analyzed audience.

Learning formats and media extended beyond formal journals.

The source analysis also identified clinical-trial content, TEDMED, TED Talks, news outlets, radio, newspapers, blogs, digital products, and apps. This broader media layer was useful because professional learning does not happen only inside journals. Clinicians may encounter a topic through a conference discussion, an institutional post, a colleague, a news story, or a digital platform before seeking the underlying evidence.

For launch planning, the practical implication was to separate discovery channels from evidence channels. A broad media source may create awareness, while a society, journal, peer, or institution may provide the credibility needed for deeper consideration.

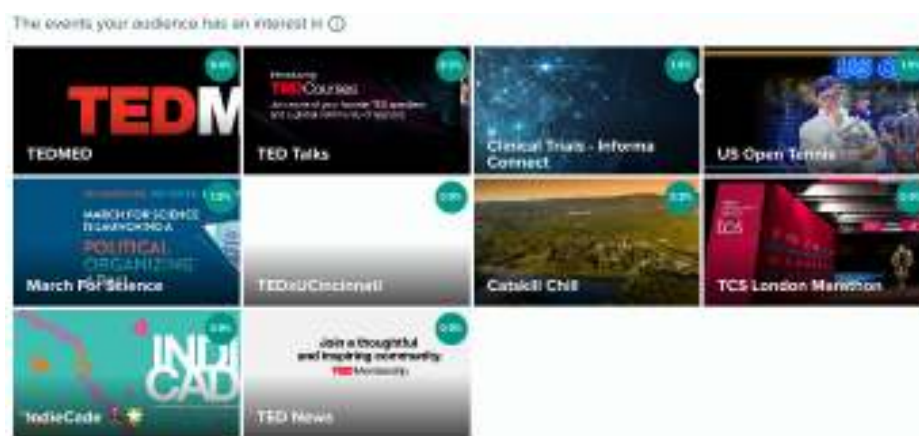


Figure 8. Selected event and learning-format affinities.



Figure 9. Television and news affinities.



Figure 10. Newspaper affinities.

Three professional contexts organized the audience without ranking it.

The professional segmentation reflected the contexts requested for the study. It did not label clinicians as better or worse and did not require one person to belong to only one category. Its purpose was to make differences in evidence needs and implementation conditions easier to discuss.

Professional context	What the research examined	Potential launch question
Large clinical centers	Connections to major systems, multidisciplinary environments, and visible institutional pathways.	What evidence and operational review would a large system require?
Major academic publishers	Publication activity, research topics, terminology, and participation in scientific discussion.	Which clinical claims and study details will withstand expert scrutiny?
Small and midsize hospitals	Practice context, implementation realities, resource constraints, and workflow considerations.	What would make adoption practical outside a major academic center?

How the segmentation could be used

- Recruit a more varied mix of clinicians for interviews, advisory work, or message testing.
- Prepare discussion guides that reflect different institutional and operational realities.
- Avoid overbuilding a launch strategy around the most publicly visible academic voices.
- Compare whether the same value proposition remains credible across contexts.

Because the categories may overlap, they are best used as sampling and interpretation lenses rather than as a rigid market taxonomy.

Public clinical footprints revealed recurring differences in what mattered.

The qualitative review examined public materials from 16 physicians, including publications, interviews, congress content, institutional pages, and online discussion. Across those materials, the analysis identified recurring themes related to patient-centered interaction, breast-conserving surgery, survival, new technology, guideline validity, positive margins, real-time devices, and three-dimensional data.

Three personas were developed as interpretive summaries. They are not demographic profiles and were not measured as shares of the 106-profile audience. Their value lies in helping a team anticipate different questions before direct research or activation.

Persona	Primary orientation	Questions likely to matter
Tech-driven effectiveness	Evidence, new tools, margin assessment, and reducing re-excision.	How well does it perform? What does the evidence show? How does it improve intraoperative decision-making?
The “Just Right” approach	Individualized treatment, tissue preservation, aesthetics, symmetry, and oncoplastic care.	Can it support appropriate margins while protecting the patient experience and surgical outcome?
Strict guidelines	Safety, established standards, practical clarity, education, and implementation support.	Where does it fit within accepted practice? What training, workflow, and safeguards are required?

The personas created a vocabulary for planning. They did not replace direct interviews with clinicians or evidence from real product use.

Different postures toward innovation require different forms of proof.

Tech-driven effectiveness

This persona is oriented toward measurable improvement and the practical potential of new technology. Public materials emphasized evidence, margin assessment, reducing re-excision, and tools that could improve clinical decision-making. Academic centers, teaching environments, papers, webinars, interviews, and professional social content were relevant parts of the surrounding information environment.

The “Just Right” approach

This persona emphasized individualized treatment and balance. The clinical conversation included tissue preservation, aesthetics, symmetry, oncoplastic approaches, and margins tailored to the patient and procedure. A useful value proposition would need to connect clinical performance with the broader surgical and patient experience.

Strict guidelines

This persona prioritized safety, established guidance, practical clarity, education, and implementation support. Interest in innovation did not eliminate the need to understand how a new approach fit within existing practice. Evidence communication would need to reduce ambiguity and address training, workflow, and appropriate use.

A strong launch strategy does not choose one persona and ignore the others. It creates a common evidence foundation, then adapts emphasis and format to the questions different clinicians bring.

The research created hypotheses for content, channels, and validation.

The study was most useful when treated as a decision tool rather than a static audience portrait. It identified where the team could be more precise and where additional direct research was still needed.

Decision area	What the evidence suggested	Recommended next validation
Message architecture	Connect technology to clinical performance, patient experience, and implementation.	Concept and message testing with clinicians from all three professional contexts.
Evidence strategy	Offer different levels of scientific and operational depth.	Review claims and supporting evidence with academic and practice-based HCPs.
Channel planning	Professional societies, specialist voices, institutions, journals, events, and digital media may play different roles.	Test reach, engagement, and response by source rather than assuming affinity equals performance.
Research recruiting	Professional context matters alongside specialty.	Use segmentation to diversify interview and advisory samples.
Content formats	Papers, webinars, interviews, congress content, and implementation materials can serve different needs.	Match formats to stage of decision and measure downstream action.
Measurement	Digital affinity is a hypothesis, not an outcome.	Track observed engagement, qualified response, and progression through the launch funnel.

A practical sequence

Profile → Recruit → Test → Activate → Measure. Use digital profiling to structure the audience, recruit direct research participants, test the value proposition, activate a limited channel plan, and update the model with observed behavior.

The evidence is useful when its boundaries remain visible.

Boundary	What it means for interpretation
Historical snapshot	The case reflects research conducted in 2022. Professional conversations, platforms, and audience behavior change over time.
Selected digital audience	The 106 profiles were selected for strategic relevance and are not a representative sample of all US oncologists.
Platform estimates	Demographics, interests, and affinities are modeled estimates and may be incomplete or inaccurate.
Public footprint	Professionals with limited online activity may still have substantial clinical or institutional influence.
Network relationships	Following and follower links indicate visible connection, not agreement, endorsement, or offline collaboration.
Overlapping segments	Professional contexts and affinity clusters may overlap and should not be treated as exclusive categories.
Qualitative personas	Personas summarize recurring themes. They do not establish prevalence or predict individual behavior.
No outcome claim	The case describes research inputs for launch planning and does not claim campaign uplift, adoption, or clinical outcomes.

Appropriate use

This report is appropriate for research design, audience strategy, content planning, and hypothesis generation. It should not be used as clinical guidance, an assessment of clinician quality, or evidence that any named individual or organization endorsed a product.

Research and analytics for healthcare decisions.

HealthLife Metrics helps healthcare teams understand markets, patients, caregivers, healthcare professionals, staff, digital experiences, and performance data. The work is designed around a practical question: what evidence will make the next decision clearer?

Relevant capabilities

Capability	Examples
Healthcare market intelligence	Market assessment, competitive intelligence, landscape research, opportunity framing
HCP and staff research	Interviews, digital profiling, segmentation, message testing, workflow and evidence-needs research
Patient and caregiver research	Needs, journeys, barriers, decision drivers, concept testing, qualitative and quantitative research
Healthcare UX research	Usability testing, information architecture, accessibility, product experience, conversion barriers
Healthcare data analytics	Measurement planning, dashboards, behavioral analysis, segmentation, decision support

Discuss a project

healthlifemetrics.com/#quote

Read the online case study

healthlifemetrics.com/hcp-staff-research.html

Client and device names are withheld. This report is intended for research and strategy discussion and does not provide clinical guidance.